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APPENDICIES

Appendix A : Themed Interview Questions on Privacy Governance

Theme	Respondent Role	Key Interview Questions
1. Governance and Leadership in Privacy Risk	Hospital Director	Who is accountable to determine the strategic risk level? What is the top management strategy in oversees privacy governance? Why is privacy risk not included in the hospital's strategic priorities? What barriers in formalizing a Data Protection Officer Role? How is the top management managing privacy governance across leadership roles?
	Quality Committee	How does the committee responsible in formulating or update of privacy-related SOPs? What is the relation between quality audits and data protection assessments? What challenges are faced when updating SOPs for privacy risk governance? How aware is the committee of current legal obligations for SIMRS?
	IT Lead	What IT mechanism for privacy risk in SIMRS? Who monitors SIMRS data access? How is IT involved in strategic decision about data protection? If What is the IT in case of data violation? What structure could support IT for privacy monitoring in SIMRS?
	Medical Records Officer	How crucial do the medical records unit in supporting privacy governance? How are SOPs communicated and enforced within the medical record department?

		How dependent does a patient's data privacy handle in medical records issues? Is there clear delegation for data privacy responsibilities between records and IT teams?
2. Privacy Risk Identification and Assessment	Hospital Director	Has privacy risk been formally registered in hospital's privacy register? What is the top management of hospital identified and mitigate the privacy risks? How did each of the division in handling privacy risk? How steps can be done in reducing third-party dependence for SIMRS?
	Medical Records Officer	Have privacy risks in patient's record handling ever been formally identified? Are there privacy assessments of physical medical records into electronic medical records changes? How are internal incidents recorded and escalated? Is there documentation of potential patient data exposure?
	Ministry of Health – Data Transformation Office (DTO)	What are the typical gaps found in terms of digital maturity in health sector? Do the Ministry of Health provide templates or training for privacy risk or data breach assessment? What is the minimum requirement in terms of data protection and handling data privacy governance in health sector? How can ministry of health support privacy governance in regional/smaller health sector?
	Quality Committee	What assessment method is used to identify non-clinical risks including privacy? How can Quality Committee assure the data privacy policy between IT and Medical Records?

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		How will privacy risk be formally added to the risk register?
3. Privacy Controls and Operational Safeguards	Medical Records Officer	How is patient consent collected and digitized in SIMRS? What are the current challenges in managing access to EMRs?
	IT Lead	Does IT do data trail for log access to patient records? What technical upgrades are needed to improve traceability and compliance? Are there any data process or data handling restriction on SIMRS?
4. Monitoring and Incident Management	IT Lead	Are there SOPs for incident related to data privacy risk? What tools or alerts should there be for data privacy risk? Are there any data impact assessments for data privacy risk?
	Quality Committee	How can be improved for data governance readiness? Are there any data impact assessment or data routine drills or audits? How does the committee validate the effectiveness of existing data privacy SOPs in terms of PDP Law?
	Medical Records Officer	What is the typical response workflow when data breach involves medical records? Is there any routine coordination with IT teams regarding electronic medical records (EMR) ? What permissions are needed to adjust or retrieve EMR in SIMRS? Are patients informed of incidents impacting their data?
5. Legal and Regulatory Compliance	Ministry of Health	Which clause should be included for SIMRS vendor agreement?

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		What are common misconceptions among hospitals regarding legal obligations? How is the ministry enforcing PDP readiness in the health sector?
	Hospital Director	What resources are required to appoint a DPO? What is the hospital's plan to implement PDP Law especially in highly data processing department? Are there any legal review of data sharing and retention policies in the SIMRS agreement?

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Appendix B : ROPARI Hospital Risk Management Policy

1. Table 1 - Severity of Non-Clinical Risk
   In this table, it also can be inferred that Ropari Hospital did not include data privacy as non-clinical risk.

Level Dampak	Area Dampak
Sangat Rendah (1)	Tidak berdampak pada pencapaian tujuan intansi/kegiatan secara umum
	Agak mengganggu pelayanan
	Dampaknya dapat ditangani pada tahap kegiatan rutin
	Kerugian kurang material dan tidak mempengaruhi stakeholders
Rendah (2)	Mengganggu pencapaian tujuan intansi/kegiatan meskipun tidak signifikan
	Cukup menggangu jalannya pelayanan
	Mengancam efisiensi dan efektivitas beberapa aspek program
	Kerugian kurang material dan sedikit mempengaruhi stakeholders
Sedang (3)	Mengganggu pencapaian tujuan intansi/ kegiatan secara signifikan
	Mengganggu kegiatan pelayanan secara signifikan
	Mengganggu administrasi program
	Kerugian keuangan cukup besar
Tinggi (4)	Sebagian tujuan intansi/kegiatan gagal dilaksanakan
	Terganggunya pelayanan lebih dari 2 hari tetapi kurang dari 1 minggu
	Mengancam fungsi program yang efektif dan organisasi
	Kerugian besar bagi organisasi dari segi keuangan maupun non keuangan
Sangat Tinggi (5)	Sebagian besar tujuan intansi/kegiatan gagal dilaksanakan
	Terganggunya pelayanan lebih dari 1 minggu
	Mengancam program dan organisasi serta stakeholders
	Kerugian sangat besar bagi organisasi dari segi keuangan maupun non keuangan

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2. Table 2 –Example of hospital risk-based service
   In this table data privacy is also not included as the area risk of procedure to maintaining hospital's risk.

No	Area Risiko
1	Akses Pasien : 1. Proses pemulangan pasien lama 2. Pasien pulang paksa
	3. Kegagalan merujuk pasien 4. Ketidaktersedianya tempat tidur 5. Proses trensfer pasien yang tidak baik
2	Kecelakaan : 1. Tersengat listrik 2. Terpapar dengan bahan berbahaya 3. Tertimpa benda jatuh 4. Tersiram air panas 5. Terpeleset
3	Assesment dan Terapi 1. Kesalahan identifikasi pasien 2. Reaksi transfusi darah 3. Kesalahan pelabelan spesimen laboraturium 4. Kegagalan konsultasi interdisiplin pasien 5. Code Blue
4	Masalah administrasi keuangan pasien 1. Kesalahan estimasi biaya 2. Pengenaan tagihan yang sama 2X 3. Kesalahan inpit data tagihan 4. Perbedaan tarif dan tagihan 5. Transaksi tidak terinput
5	Kejadian infeksi 1. Kegagalan / kontaminsai alat medis 2. Infeksi operasi 3. Needlestick injury 4. Kesalahan pembuangan limbah medis 5. Infeksi nosokomial
6	Rekam medik 1. Kegagalan memperoleh informed consent 2. Kesalahan pelabelan rekam medik 3. Kebocoran informasi rekam medik 4. Ketidaklengkapan catatan dalam rekam medik 5. Kehilangan/ kesalahan penyimpanan rekam medik
7	Obat : 1. Penulisan resep yang tidak baik 2. Riwayat alergi obat tidak teridentifikasi 3. Kesalahan dosis obat 4. Obat rusak/ expired 5. Kesalahan identifikasi pasien dalam pemberian obat 6. Kegagalan memonitor efek samping obat
8	Keamanan 1. Pencurian 2. Pasien hilang 3. Lingkungan yang tidak aman

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Appendix C : Ropari Hospital Medical Records and IT SIMRS Technical Requirements

There are medical records retention requirements and IT SIMRS procedure but they are not focusing on personal data protection requirements.

1. Table 3 – Physical Medical Records Retention Flow



2. Table 4 - IT SIMRS Downtime Procedure

PROSEDUR	a. Downtime terencana 1. Petugas IT/SIMRS akan meminta izin untuk mematikan server 2. Petugas IT/SIMRS akan memberitahukan bahwa aka nada perbaikan terkait server (maintenance) di grup Rumah Sakit 3. Apabila perbaikan (maintenance) telah selesai, maka akan diinfokan ulang kepada grup Rumah Sakit b. Downtime tidak terencana 1. Gangguan Penyedia Layanan Internet 1) Petugas IT/SIMRS mendapat laporan dari penyedia bahwa ada maintenance jaringan internet 2) Petugas IT/SIMRS memberitahukan pada grup bahwa ada downtime Gangguan dari ISP 3) Apabila internet sudah normal kembali, petugas melakukan konfirmasi ulang/memberikan informasi pada grup 2. Ganguan Komputer/Software/Jaringan 1) PEtugas IT/SIMRS mendapat laporan dari unit terkait bahwa ada komputer yang tidak bisa akses SIMRS 2) Petugas SIMRS melakukan pengecekan terkait masalah tersebut 3) Petugas IT/SIMRS memberitahukan masalah pada Ka Unit
	4) Petugas IT/SIMRS melakukan perbaikan terkait kendala yang ditemukan 5) Apabila sudah normal kembali, Petugas IT/SIMRS akan mengkonfirmasi kalau komputer sudah terhubung kembali

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Appendix D - Ropari Hospital Legal and Compliance Requirements

Ropari Hospital vendor's dependency is also followed by the absence of data protection and data processing clause

1. Table 5 – IT SIMRS Vendor's Agreement Rights and Obligations Clause

PASAL 5

HAK DAN KEWAJIBAN

1. HAK DAN KEWAJIBAN PIHAK PERTAMA

a) Berkewajiban membentuk tim internal Rumah Sakit yang ditempatkan khusus untuk project ini, yang terdiri dari:
   a.1. Project Champion (PIC)
   a.2. Tim Tarif
   a.3. Tim Master Data
   a.4. Tim Keuangan.

b) Mendapatkan HAK untuk menggunakan Sistem Informasi Manajemen Rumah Sakit (SIMRS) dalam bentuk perangkat lunak (Aplikasi) SIMRS Pandeka

c) Berhak mendapatkan servis, pemeliharaan dan pemutakhiran informasi terbaru selama jangka waktu perjanjian ini.

d) Mendapatkan manual Book dari **PIHAK KEDUA** sebagai panduan penggunaan perangkat lunak Aplikasi SIMRS Pandeka

e) Berhak mendapatkan pemenuhan *Service Level Agreement (SLA)* sebagai berikut:
   d.1. Laporan diterima dan direspon selambat-lambatnya 3 jam sejak dilaporkannya isu di *system ticketing*

   d.2. Kategori tiket ditentukan oleh *representative* **PIHAK PERTAMA** yang terdiri dari:

      e.2.1 **Level 1**, Petunjuk Proses Alur Bisnis, Petunjuk guna. Dan direspon selambat-lambatnya 2 jam setelah dilakukan kategorisasi.
      e.2.2 **Level 2**, Perubahan Data, Penyesuaian Data Historis, Bugs, Error. Di respon selambat-lambatnya 6 jam setelah dikategorisasi.
      e.2.3 **Level 3**, Perubahan Source Code aplikasi karena adanya penambahan atau perubahan fitur. Disespon selambat-lambatnya 12 jam setelah dilakukan kategorisasi.

f) Berkewajiban menandatangani berita acara serah terima (BAST) dan/atau Berita Acara Perkembangan Progres (BAPP) secara objektif

g) Berkewajiban melakukan pembayaran dari Kerjasama ini sesuai dengan mekanisme yang disepakati pada pasal 4 perjanjian ini

h) Berkewajiban menanggung biaya implementasi apabila melebihi jangka waktu yang disepakati.

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2. **HAK DAN KEWAJIBAN PIHAK KEDUA**

a) Berkewajiban untuk melakukan pendampingan pada saat Perencanaan, Monitoring dan Sosialisasi kepada tim **PIHAK PERTAMA** sampai dinyatakan siap untuk bekerja tanpa monitoring (*User Friendly*).

b) Berkewajiban untuk datang ke lokasi jika mendapatkan pelaporan trouble aplikasi dari **PIHAK PERTAMA** dan tidak bisa diperbaiki dengan *system remote*.

c) **PIHAK KEDUA** Berkewajiban memberikan hak guna dan hak milik perangkat-perangkat lunak Aplikasi SIMRS Pandeka setelah selesai 2 (dua) tahun masa KSO. Untuk tata laksana lanjutan setelah KSO akan dibicarakan kemudian.

d) **PIHAK KEDUA** berkewajiban melakukan pendampingan pada Tim IT **PIHAK PERTAMA** saat proses sosialisasi dan transfer ilmu selama proses KSO berlangsung

e) **PARA PIHAK** bekerjasama memaksimalkan *timeline* yang telah disepakati sebagai percepatan dalam menggunakan Sistem Informasi Manajemen Rumah Sakit (SIMRS) dalam bentuk perangkat lunak (Aplikasi) SIMRS Pandeka

f) Berkewajiban menjamin service, pemeliharaan, pemutakhiran informasi terbaru selama jangka waktu perjanjian Kerjasama ini.

g) Berhak menerima pembayaran dari Kerjasama ini sesuai dengan mekanisme yang disepakati pada pasal 4 perjanjian ini.

h) Berhak menerima data yang diperlukan untuk proses implementasi perangkat lunak Aplikasi SIMRS

i) Berkewajiban menyelesaikan spesifikasi fitur yang dimiliki perangkat lunak aplikasi SIMRS Pandeka yang terdapat pada lampiran IV dalam perjanjian ini.

j) Berkewajiban menyelesaikan Implementasi dengan jangka waktu yang disepakati selama infrastruktur **PIHAK PERTAMA** mendukung.

k) Berkewajiban mendukung membantu dan mempermudah **PIHAK PERTAMA** dalam mengimplementasikan Aplikasi SIMRS